must not rest solely on the presence of "retinal hemorrhage" in either the clinical or the research setting.[2] In addition to describing the type, location, distribution pattern, and number of retinal hemorrhages, one must also take into consideration the history (or absence of history) and the nature of other injuries, particularly to the bones and brain.

The medical problem of child abuse is discussed in a unique medicolegal context, so much so that some physicians have virtually made their careers out of testifying as expert witnesses. Legal and medical definitions of science and fact can be disturbingly different. Clarity of definition and adherence to proper medical scientific principles of investigation, including peer-review publication, remain an essential element in exploring and understanding the pathophysiology and potential forensic implications of findings such as retinal hemorrhage.

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Macular Hole Surgery

Dear Editor:

We greatly appreciate the reply to our letter by Dr. Da Mata and colleagues[1] and fully agree that more comparative data on functional outcome of indocyanine green (ICG)–assisted macular hole surgery are necessary. Therefore, our clinical and histopathologic results have been submitted for publication.

From a clinicopathologic point of view, one comment by Dr. Da Mata and colleagues needs further clarification. They used the term "surface of retinal ILM," which might be misleading, as the interpretation of histologic findings requires further information on which side of the internal limiting membrane (ILM) the authors refer to, the retinal or the vitreal side. When evaluating the ultrastructure of the ILM removed during vitrectomy, the key questions are the following:

1. Are cellular elements (such as inner portions of Müller cells) present on the retinal surface of the ILM?
2. If so, can this be attributed to ILM peeling itself or to the additional use of ICG?

In their reply Dr. Da Mata and colleagues mention the presence of cellular elements "on the surface of retinal ILM" in eyes with "coexisting epiretinal membranes." In these cases, it should not be surprising to find cellular elements on the vitreal side of the ILM, as shown by previous reports as well as by Dr. Da Mata and coworkers themselves.[2]

Regarding the retinal side of the ILM, there were no cellular elements present after ICG-assisted peeling, as shown in Figure 2 of the latter report.[2] In our experience, the absence of cellular elements on the retinal side of the ILM is consistent with ultrastructural analysis of specimens after ILM peeling, but *without* ICG assistance. In contrast, after ICG-assisted ILM removal, cellular elements of the inner retina adherent to the ILM were common features of histologic specimens.[3] In a previous study these findings were also observed in cadaver eyes but were considered a postmortem artifact.[4] The presence of cellular structures adherent to the ILM was further reported by Kwok and colleagues after ICG-guided vitrectomy.[5] However, from the figures provided in their report and due to the histologic technique applied, it was hardly possible to determine the real location of these cells.[6]

In summary, we emphasize that retinal structures had been found on the retinal surface of the ILM after ICG-assisted vitrectomy not only in cadaver eyes but also after ICG-guided macular hole surgery in humans. Attention should be paid to ultrastructural analysis of ILM specimens after ICG-assisted peeling, as this might further clarify recent controversial discussions on possible negative effects of ICG on the human retina.

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Postoperative Wound Dehiscence

Dear Editor:

The article by Abou-Jaoude et al[1] brings to mind a postoperative complication after penetrating keratoplasty (PKP) that is rarely investigated. In this retrospective, noncomparative study of 69 eyes that underwent removal of a single running suture after PKP, the authors identified 5 that de-

veloped spontaneous wound dehiscence within the first 2 weeks after suture removal, a 7.2 % complication rate. They further concluded that patients undergoing PKP for corneal edema and patients receiving prolonged corticosteroid treatment are at risk of developing this complication.

We agree that such a high complication rate is not acceptable. Closer attention for prevention is therefore necessary, given the fact that not only significant astigmatism but also a number of more severe complications may occur, particularly in patients with traumatic alteration after PKP.[2] Although it is difficult to obtain accurate data on incidence and the risk factors for this complication, a careful examination of related causes may be helpful.

Our experience indicates that the population of keratoconus patients is also a risk group. Keratoconus is the leading indication for PKP in many clinical centers and is often characterized by an altered wound-healing response.[4–6] Commonly, these are younger patients who undergo corneal grafting (mean age at our institution = 43 years) and are particularly severely affected because they are often still working. In these patients, the suture technique is of particular importance, especially regarding suture loosening and related complications. This brings us to our second issue.

As correctly cited in the Discussion, a previous study did not find a relationship between wound dehiscence and the suture technique.[7] However, it is our experience, shared by other authors,[8,9] that the suture technique is an important factor and key to lowering the incidence of postoperative wound dehiscence. In these patients we proceed with a step-by-step approach that includes careful preoperative evaluation of the corneal topography, routine use of double running sutures (10-0, plus 11-0 nylon), and close follow-up, including corneal topography, as well as the limited use of topical corticosteroids. Removal of sutures is delayed 1 year postoperatively and only performed in patients with $>$4.5 diopters or noncorrectable irregular astigmatism. If suture removal seems to be indicated, a two-step approach is used: initial removal of the 11-0 suture after topographic study, followed by serial mapping to detect changes in corneal curvature. In the case of significant changes, particularly in the steeper axis, we wait even longer before removal of the remaining suture. By practicing this procedure, we have less than a 2% dehiscence rate.

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